Clinical Study Summary Report (CSSR) Document Status: Draft | **Version:** 1.0 | **Date:** 03-Apr-2026

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1. Administrative & Study Identification

Full Study Title: A Phase 1 Open-label, Multicenter Study to Evaluate the Safety, Tolerability, Pharmacokinetics, and Antitumor Activity of SGN-MesoC2 in Subjects With Advanced Solid Tumors **Short Title/Acronym:** A Study of SGN-MesoC2 in Advanced Solid Tumors **Protocol Number:** PF-08052666 **NCT Number:** NCT06466187 **Sponsor Name:** Seagen, a wholly owned subsidiary of Pfizer **Investigational Product:** SGN-MesoC2 / PF-08052666 / MesoC2 (Antibody-Drug Conjugate targeting MSLN) **Phase of Development:** Phase I **Medical Monitor:** Pfizer Clinical Operations Lead

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2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To evaluate the type, incidence, and severity of adverse events (AEs), incidence of dose-limiting toxicities (DLTs), and to assess the overall safety, tolerability, and pharmacokinetics of SGN-MesoC2 in patients with advanced solid tumors across dose escalation, optimization, and expansion cohorts. **Secondary Objectives:** To evaluate preliminary antitumor activity including Objective Response Rate (ORR), Duration of Response (DOR), Disease Control Rate (DCR), Progression-Free Survival (PFS), and Overall Survival (OS) per RECIST v1.1 criteria, as well as MSLN expression and changes in tumor-specific biomarkers.

2.2 Background & Rationale MesoC2 (PF-08052666) is a novel antibody-drug conjugate (ADC) composed of a fully human IgG1 anti-mesothelin (MSLN) monoclonal antibody conjugated to a topoisomerase 1 inhibitor (TOP1i) payload. MSLN is a tumor-associated antigen highly upregulated in various solid tumors, with limited expression in normal tissues. Advanced, platinum-resistant, or refractory solid tumors—such as mesothelioma, ovarian, pancreatic, and non-small cell lung cancer—represent an area of high unmet medical need. This Phase 1 study will define the safety profile, maximum tolerated dose (MTD), and recommended dose for expansion of SGN-MesoC2.

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3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional **Control Type:** Single-arm (Dose escalation, optimization, and expansion) **Blinding:** Open-label **Randomization:** N/A

3.2 Planned Enrollment & Duration Target N: Up to 365 participants (45 in dose escalation, 40 in dose optimization, 280 in dose expansion) **Number of Sites:** Multicenter (Global) **Estimated Study Start:** Mid-2024 **Estimated Primary Completion:** [Pending]

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4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Aged 18 years or older. 2. Histologically- or cytologically-confirmed metastatic or locally advanced unresectable platinum-resistant ovarian cancer (PROC), non-small cell lung cancer (NSCLC), pancreatic ductal adenocarcinoma (PDAC), endometrial cancer (EC), colorectal cancer (CRC), or mesothelioma. 3. Patients who have relapsed or progressed following standard therapies, or for which no standard therapies are available. 4. Eastern Cooperative Oncology Group (ECOG) performance status score of 0 or 1. 5. Available archival tumor tissue or willingness to provide a fresh biopsy.

4.2 Exclusion Criteria 1. Prior or current treatment with systemic anticancer therapy or focal radiotherapy within 4 weeks prior to the first dose of MesoC2. 2. Prior anti-MSLN targeted therapies. 3. Any unresolved toxicities from prior therapy greater than Grade 1 at the time of starting study treatment (except alopecia).

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5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: SGN-MesoC2 administered via dose escalation levels, followed by dose/ schedule optimization, and fixed doses in the disease-specific expansion phase. **Route of Administration:** Intravenous (IV) Infusion. **Duration of Treatment:** Continued until disease progression (PD) according to RECIST v1.1, unacceptable toxicity, or patient withdrawal.

5.2 Concomitant & Prohibited Therapy Permitted: Standard supportive care medications for the management of therapy-related toxicities (e.g., antiemetics). **Prohibited:** Concurrent administration of any other investigational agents, systemic anticancer therapies, or prior anti-MSLN therapies.

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6. Study Timeline & Visit Schedule

| Visit ID | Window (Days) | Key Activities/Procedures | | :--- | :--- | :--- | | **Screening** | Day -28 to 0 | Informed Consent, Medical History, Tumor Biopsy (if archival not available), Baseline Imaging | | **Baseline** | Day 1, Cycle 1| First IV Dose, PK/PD blood sampling, Safety Baseline Labs | | **Interim Visits** | Q3W/Q4W | Safety Labs, PK/PD monitoring, Efficacy Assessments (CT/MRI every 6-8 weeks) | | **End of Study** | 30 days post-last

dose | Final Vital Signs, Exit Interview, Radiographic confirmation of PD, Safety Follow-up |

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7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Definition: Type, incidence, and severity of adverse events (AEs), frequency of dose modifications due to AEs, and incidence of dose-limiting toxicities (DLTs) to establish safety and determine the recommended Phase 2 dose. **Measurement Method:** Evaluated using the Common Terminology Criteria for Adverse Events (CTCAE) version 5.0.

7.2 Secondary & Exploratory Endpoints * **Endpoint 1:** Objective Response Rate (ORR), Duration of Response (DOR), Disease Control Rate (DCR), and Progression-Free Survival (PFS) assessed per RECIST v1.1. * **Endpoint 2:** Pharmacokinetics (PK), MSLN expression in blood and tissue, and changes in tumor-specific biomarkers.

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8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Continuous monitoring of AEs and serious adverse events (SAEs) collected via clinical assessment at every study visit. **Serious Adverse Events (SAEs):** Reported to the sponsor within 24 hours of investigator awareness. **Data Monitoring Committee (DMC):** Internal Dose Escalation Committee (DEC) to review safety, PK, and DLT data prior to dose escalation.

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9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: * **Safety Set:** All patients who receive at least one dose of SGN-MesoC2. * **Efficacy Evaluable Set:** Patients who receive at least one dose, have measurable baseline disease, and at least one post-baseline scan. **Sample Size Justification:** Approximately 365 patients; the dose-escalation phase is powered to identify DLTs using standard escalation rules, while the expansion cohorts (approx. 280 patients) are powered to detect early efficacy signals in specific tumor types. **Handling of Missing Data:** Standard oncology guidelines; time-to-event variables (PFS, OS, DOR) will be censored at the last known date of adequate assessment.

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10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki. **Monitoring Plan:** Routine onsite and remote monitoring visits to review safety data, query eCRFs, and ensure protocol adherence.

Audit Trail: Validated Electronic Data Capture (EDC) system with full audit trails for eCRF locking and data clarification forms.

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11. Study Identifiers & Governance

Primary ID: PF-08052666 **Registry Number:** NCT06466187 **Sponsor/Lead Organization:** Seagen, a wholly owned subsidiary of Pfizer **Study Title:** A Study of SGN-MesoC2 in Advanced Solid Tumors **Official Regulatory Title:** A Phase 1 Open-label, Multicenter Study to Evaluate the Safety, Tolerability, Pharmacokinetics, and Antitumor Activity of SGN-MesoC2 in Subjects With Advanced Solid Tumors

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12. Clinical Framework (Oncology Specific)

Primary Condition: Advanced Solid Tumors (Mesothelioma, Ovarian, NSCLC, Pancreatic, Endometrial, Colorectal) **Diagnosis Threshold:** Histologically/cytologically-confirmed advanced unresectable or metastatic disease **Medical Methodology:** Topoisomerase 1 inhibitor (TOP1i) payload delivered via Mesothelin-Targeted Antibody-Drug Conjugate (ADC) **Optimization Target:** Maximum Tolerated Dose (MTD) / Recommended Phase 2 Dose (RP2D)

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13. Study Procedures & Methodology

Management Pathway: Phase 1 Oncology Dose Escalation and Cohort Expansion **Education Protocol (HCP):** Site initiation visits, protocol training, and DLT management criteria **Education Protocol (Patient):** Informed consent review, study visit adherence, and AE reporting education **Quality Audit Mechanism:** Centralized data review, clinical site monitoring, and safety review committees

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14. Observation & Follow-Up Schedule

Total Duration: Time from first dose to disease progression, unacceptable toxicity, or death (OS follow-up) **Onsite Visit Cadence:** Every treatment cycle (typically every 3 to 4 weeks depending on the specific dosing schedule being evaluated) **Remote Monitoring Frequency:** Routine data management reviews per protocol specific monitoring plan **Checklist Review Interval:** Efficacy scans assessed every 6-8 weeks

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15. Sign-off & Approval

| Role | Name | Signature | Date | | :--- | :--- | :--- | :--- |
| **Principal Investigator** | [Name] | ____ **| [DD-MMM-YYYY] | |**

Clinical Operations Lead | [Name] | ___ | ***[DD-MMM-YYYY]* | |** ***Lead Statistician | [Name] |*** _____ | [DD-MMM-YYYY] |